

Item Code	: 204287 - Draft 03 Nov 2020
PM Code	: Ins.P - BCS
Measurement(mm) $\pm$ 2mm	: 180 (L) x 150 (W)
Colour of Artwork	: K
Source of Artwork	: MAC 2 / CC

BUCANIL[®] SYRUP

Ingredient(s):

Each ml contains:
Terbutaline Sulfate 0.3 mg

Preservative(s):

Methyl Paraben 1.8 mg
Propyl Paraben 0.2 mg

Pharmacodynamic:

Terbutaline Sulfate is a direct-acting sympathomimetic agent with action and uses similar to those of Salbutamol. Terbutaline Sulfate is a β_2 -adrenergic receptor agonist which has been shown in vitro and in vivo pharmacologic studies in animals to exert a preferential effects on β_2 -adrenergic receptors, such as those located in bronchial smooth muscle.

Pharmacokinetics:

- Terbutaline is a selective β_2 -adrenergic agonist with effects on smooth and skeletal muscle. These include bronchodilation, relaxation of uterine muscle, vasodilation and tremor.
- Terbutaline has been shown in controlled clinical studies, by activation of adenylyl cyclase, to relieve bronchospasm in chronic obstructive pulmonary disease such as asthma, chronic bronchitis and emphysema.
- A measurable change in pulmonary function usually occur within 30 minutes following oral administration. The maximum effect usually occur within 2 ~ 3 hours. There is a clinically significant decrease in airway and pulmonary resistance which persists for at least 1 hour or longer in most patients.
- After single oral dose, peak concentrations are found 30 minutes to 5 hours after administration. Most of the absorbed oral dose is metabolized via the liver. Terbutaline has a half-life of 3 ~ 4 hours and is excreted in the urine.

Indication(s):

For the relief of reversible bronchospasm, asthma, bronchitis and emphysema.

Dosage and Administration:

Adult : 10 or 15ml, 3 times a day, administered at approximately 6 hours intervals. Maximum 50 ml/day.
Children : 0.075mg(0.25ml)/kg body weight 3 times in 24 hours period.
7-15 years : 5-10ml, 3 times in 24 hours.

Route of Administration:

Taken orally.

Contraindication(s):

Bucanil[®] is contraindicated in patients with a history of hypersensitivity to any of its component or sympathomimetic amines.

Side Effect(s) / Adverse Reaction(s):

The most common adverse effect are nervousness, restlessness and trembling. Other adverse reactions include sweating, increase in blood pressure, nausea, vomiting, drowsiness, dizziness or lightheadedness, increased heartbeat and headache. The frequency of these side effects appear to diminish with continued therapy.

Precaution(s) / Warning(s):

- Should be used with cautions in patients with cardiovascular disorders, in patients with hyperthyroidism or diabetes mellitus, history of seizures, or in patients who are unusually responsive to sympathomimetic amines.
- Problem of terbutaline in human have not been documented. However, terbutaline is excreted in breast milk and some animal studies have shown this medication to be potentially tumorigenic.
- Although no teratogenic effects have been observed in animals or in humans, caution is recommended during 1st trimester of pregnancy.
- Proper use of this medication that follow the recommended amount and interval is important to avoid patient exposure to the side effects and overdosage of terbutaline.
- Tocolysis: Serious adverse reactions including death have been reported after administration of terbutaline to women in labor. In the mother, these include increased heart rate, transient hyperglycaemia, hypokalaemia, cardiac arrhythmias, pulmonary oedema and myocardial ischaemia. Increased fetal heart rate and neonatal hypoglycaemia may occur as a result of maternal administration.

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($\pm$ 2mm)

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($\pm$ 2mm)

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Pregnancy & Lactation:

Terbutaline is excreted in breast milk in some animal studies have shown this medication to be potentially tumorigenic.

Drug Interaction(s):

Antihypertensive effects may be reduced when these medications are used concurrently with adrenergic bronchodilators, the patients should be carefully monitored to confirm that the desired effect is being obtained. For adrenergic bronchodilators with beta adrenergic effects, beta-blockade may antagonize the β_2 -adrenergic bronchodilating effects of terbutaline.

Concurrent use with adrenergic bronchodilators may result in additive CNS stimulations to excessive levels, which may cause unwanted effects such as nervousness, irritability, insomnia or possibly convulsions or cardiac arrhythmias, close observation is recommended. In addition to increasing CNS stimulation, concurrent use of a sympathomimetic may increase the cardiovascular effects of either or both medications and the risk of adverse effects.

Symptoms and Treatment for Overdosage, and Antidote(s):

Symptoms of overdose include severe chest pain, severe muscle cramping, trembling, continuing or severe of pounding heartbeat and unusual anxiety or restlessness.

Treatment for the patients who have taken excessive medication is emptying the stomach by means of induced emesis, or by gastric lavage.

In the unconscious patients, secure the airway with a cuffed endotracheal tube before beginning lavage. (Do not induced emesis).

Shelf-Life:

2 years from the date of manufacture.

Storage Condition(s):

Keep in a tight container. Store at temperature below 30°C. Protect from light and moisture.

Product Description and Packing(s):

A clear, colorless to yellowish solution.

Plastic bottle of 60ml, 100ml and 120ml

Plastic bottle of 4L (for export only).

BUCANIL[®] SIRAP

Bahan Aktif:

Setiap ml mengandung:

Terbutaline Sulfate 0.3 mg

Pengawet:

Methyl Paraben 1.8 mg

Propyl Paraben 0.2 mg

Indikasi:

Untuk melegakan bronkospasma berbalik, asma, bronkitis emfisema.

Dos:

Dewasa : 10 atau 15ml, 3 kali sehari diberi setiap 6 jam. Dos maxima tidak melebihi 50 ml/hari.

Kanak-kanak : 0.075mg (0.25ml)/kg berat badan tiga kali dalam jangka masa 24 jam.

7-15 tahun : 5-10ml, tiga kali dalam jangka masa 24 jam.



Manufacturer and Product Registration Holder:

Y.S.P. INDUSTRIES (M) SDN. BHD. (199001001034)

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Kawasan Perindustrian Bandar Baru Bangi,

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Ordering Line: 1 800 88 3027

Product Info: 1 800 88 3679

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Date of revision: 03 Nov 2020

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